

Care Pathway (Sample)

Acute and Subacute Low Back Pain: Staged Management and Referral Pathway — Synthetic example, staged/flowchart format

Pathway Overview

This pathway represents a care-workflow diagram, described here in sequential staged form as it would appear in the source flowchart. Each stage represents a decision node; the patient advances to the next stage only if they do not meet criteria for escalation at the current stage.

STAGE 1 - Red Flag Screen (Day 0)

Assess for: cauda equina symptoms (saddle anesthesia, new bowel/bladder dysfunction, bilateral leg weakness); suspected spinal fracture (significant trauma, or minor trauma in a patient with osteoporosis risk or age >70); suspected spinal infection or malignancy (fever, IV drug use, immunosuppression, unexplained weight loss, history of cancer). DECISION: If any present -> escalate immediately to emergency evaluation / same-day spine surgery referral. If none present -> proceed to Stage 2.

STAGE 2 - Neurologic Screen (Day 0)

Assess for signs of radiculopathy or significant neurologic deficit: positive straight-leg raise, dermatomal sensory loss, focal motor weakness, or absent/diminished reflex in a myotomal distribution. DECISION: If significant progressive motor deficit -> refer to spine surgery within 1 week. If mild radicular symptoms without progressive deficit -> proceed to Stage 3 with note of radicular findings. If no neurologic findings -> proceed to Stage 3.

STAGE 3 - Initial Conservative Management (Weeks 0-4)

Begin first-line conservative management: activity modification (avoid prolonged bed rest), over-the-counter analgesia (NSAIDs unless contraindicated), and patient education on expected favorable natural history. Formal physical therapy referral is optional at this stage and may be added if symptoms are not trending toward improvement by week 2. DECISION: Reassess at 4 weeks. If improving -> continue conservative management, no further referral needed. If not improving or worsening -> proceed to Stage 4.

STAGE 4 - Extended Conservative Management (Weeks 4-8)

For patients not improving by week 4: initiate or intensify structured physical therapy, consider short-course of muscle relaxant or neuropathic agent if radicular symptoms present, and reassess for red flags and neurologic findings (return to Stage 1/2 logic if new findings emerge). DECISION: Reassess at 8 weeks total. If improving -> continue conservative management. If not improving -> proceed to Stage 5.

STAGE 5 - Imaging and Specialist Referral (Week 8+)

Patients with persistent symptoms at 8 weeks despite adequate conservative management should undergo MRI (preferred) or CT if MRI contraindicated, and be referred to spine surgery or physiatry for specialist evaluation. Referral urgency (routine vs. expedited) is determined by imaging findings and the presence of radicular symptoms identified in Stage 2.